

Office Visit - Apr 16, 2026

with William Kelly, MD at UTH Mays Cancer Center, Medical Oncology

Notes from Care Team

Progress Notes by William Kelly, MD at 4/16/2026 11:45 AM

THORACIC-ONCOLOGY

INDICATIONS: NSCLC

DISEASE HISTORY: Karen Clayborne is a 60 y.o. female presenting with NSCLC.

Of note, no personal history of smoking.

The patient was in their normal state of health until she developed persistent cough in 4/2024. CT Chest 9/25/24 showed a dominant R hilar mass (8.8x7.5x7.7cm) occluding RML bronchus and severely narrowing the RUL and RLL bronchi as well as small R-sided pleural effusion, foci of nodular pleural-based enhancement, multiple small lower hypoenhancing liver masses (suspicious for mets) and lytic lesion along posterior right T9 and T3 vertebral bodies

9/27/24: Underwent **R chest thoracentesis**. Pathology read as **malignant cells consistent with adenocarcinoma metastatic from lung primary**. Specifically, these were positive for CK7, TTF-1, p16, negative for calretinin, CDX-2, PAX-8, GATA3. Unfortunately pleural fluid was insufficient for NGS testing.

10/18/24: Underwent TEMPUS xF+ from peripheral blood. This showed alteration in **BRCA2** (F2560fs, VAF 48.8%) and **EGFR** (inframe deletion on exon 19, VAF 1.4%), TMB 5mut/MB, VUS in NOTCH, KMT2C and SLFN11. MRI Brain 10/30/24 per chart with no evidence of malignancy.

11/13/24: Initiated **amivantamab with Lazertinib** (patient preferred chemo-free regimen) (Dr Krishna Alluri, TXO San Antonio). Mild infusion reaction on D1 that resolved on D2. CT TAP 2/8/25 showed partial response.

Unfortunately, she developed dizziness and was hospitalized. MRI Brain 4/2/25 showed solid and cystic mass in cerebellar vermis with **heterogeneous enhancing solid complex** (3.1x3x2.4cm) with moderate edema and severe mass effect on 4th ventricle and mild lateral and 3rd ventricle dilation. CT TAP showed volume loss of RLL and RML with atelectasis, moderate R pleural effusion, scattered liver hypodensities (up to 1cm), ill-defined region of hypoenhancement along pancreas uncinata, large uterine fibroid, ill-defined

sclerosis of T9. MRI Abd 4/4/25 showed moderate to large R pleural effusion and nonspecific enhancement of R hepatic lobe lesion, additional subcm high T2 foci in liver, high loss at L3 and T9 vertebral body, unremarkable pancreas. MRI Spine 4/2/25 showed neuroforaminal stenosis T8/9, 9/10 with related osseous expansion of right T9 pedicle, T9 metastasis and small T3 vertebral lesion. NSGY consultation with recommendation for outpatient SRS with possible ventricular shunt. Patient declined ventricular shunt.

4/14/25: Initiated SRS to **solitary cerebellar met** (plan for 30Gy in 5fx) with TXO (Dr. Dzuick).

4/23/25: Guardant360 from peripheral blood showed BRCA2 (suspected F2560fs germline variant) and BCOR alterations, TMB N/A, MSS, and VIS in CHEK, FGFR1, KDM4A, NOTCH.

5/7/25: Initiated **Osimertinib**. Patient refused chemotherapy or rechallenge of Lazertinib and amivantamab at this time. Guardant360 5/7/25 shows BRCA2 and BCOR.

LLE US 5/7/25 showed **DVT in peroneal vein** and she was started on rivaroxaban. Found to have **pulmonary embolism 5/30/25**. She developed an **IJ occlusive DVT 6/14/25** while off anticoagulation and then restarted apixaban 6/14/25.

MRI Brain 6/12/25 per chart with significant response to radiation.

CT TAP 7/22/25 showed **decrease in size of RML mass**, unchanged bandlike atelectasis within RML periphery, unchanged moderate loculated R pleural effusion, decreased size of mediastinal and R hilar LN, unchanged T9 vertebral body sclerotic metastasis, stable hepatic lesions (likely cysts and hemangiomas), new chronic-appearing weblike PE within LLL pulm artery.

MRI Brain 8/8/25 shows solitary enhancing met in cerebellum nodulus (1.4cm) with surrounding edema, minimal susceptibility artifact suggesting age-indeterminate blood products.

9/18/25: Started zoledronic acid. CT TAP 10/15/25 showed continued **decrease size** (2.7x2.2 from 3.6x3.6 in 2/18/25) of **RLL mass, decreased R pleural effusion**, stable mediastinal and R hilar LN, stable sclerotic T9 vertebral body with compression deformity. Patient directly admitted to UHS 1/15/26 for restaging and radiation but left AMA. She was recommended to choose and continue to follow with one oncologist for continuity and follow the recommended course of action in order to get proper treatment. CT TAP 1/23/26 with opacity, likely collapse of basilar segment RLL with rightward trachial and mediastinal shift, sclerotic in posterior T3 vertebral body (0.9cm), L subpleural nodule (0.4cm), focal hypodensity in panc head, ucinat process, likely focal fat, tiny calcific foci in gallbladder, consolidative opacities R lung base with minimal pleural effusion, unchanged L3 vertebral body irregularity.

2/4/26-2/6/26: Completed **27Gy to L frontal lesion**. Discussed at neuro-onc tumor board 2/6/26, decision made to continue to watch cerebellar vermis lesion. MRI CTL 1/29/26 showed stable mild expansile focal T9 lesion with moderate compression deformity, stable inferior T3 focal lesion (0.8cm), stable anterosuperior L3 vertebral. Admitted 3/29/26-4/7/26 to Northeast Baptist with right sided weakness, headache, SOB, R leg pain. She had reportedly

stopped her Eliquis prior to this due to nosebleeds and TCP. MRI Brain WOW con 3/29/26 confirmed multiple new brain metastasis and mild/mod hydrocephalus. Neurology consulted and patients started on aspirin and statin. Unfortunately, she developed acute hypoxemic resp failure 3/31/26 requiring high-flow NC and VQ scan 3/31/26 showed concern for b/l pulm emboli R greater than L for which she was started on heparin drip. Echo 4/1/26 without R heart strain, She was weaned off O2 4/3/26 and transitioned to back Eliquis. Her plt count decreased from 96 to 79 and then improved to 141 on 4/5/26, this was felt to be consumptive from pulm emboli and Eliquis was resumed. She had HTN during admission up to 218/121, requiring labetalol IV. Her home metoprolol was discontinued and carvedilol and losartan initiated with hydralazine for breakthrough. Diabetes was also controlled with lantus and SSI lispro and initiation of glipizide. Hospital course also complicated by metabolic encephalopathy, and AKI. At time of discharge R sided weakness had resolved. MRI Brain 4/10/26 shows **interval increase in mild heterogeneous nodular L paramedian vermis/cerebellar lesion**, interval more pronounced punctate nodular enhancement in L lateral and lower cerebellum and b/l occipital lobes, decrease in L paramedian frontal lobe enhancing lesion, interval mildly increased size of upper and mid cerebellar vermis. She saw radiation oncology 4/14/26 and noted to have mildly pressured speech and tangential thought patterns. She was offered psych support but declined. She also declined radiation.

INTERM HISTORY: Karen Clayborne presents to clinic. Denies changes in hearing, focal weakness, difficulty swallowing or speaking, changes in mood, cognition, or recent seizures. Denies fever, chills, cough, sore throat, chest pain, palpitations, nausea, vomiting, constipation, rash, or changes in urination. No melena. Admits to 7/10 headaches. Occasionally has to close one eye to better look at the computer. Admits to L arm and hand numbness which is getting worse. Occasional slurred speech, memory problems and word finding. Slight anxiety and depression. Gets DOE climbing stairs. Admits to fevers since 11/2025. Occasional diarrhea. She has also noted feet swelling on dexamethasone.

She is having more trouble walking and needs to hold on to things.

KPS 70

On dexamethasone 4mg AM one day and 2mg the next day, alternating. She has chosen this regimen.

BP 166/110, she took the carvedilol but couldn't get losartan due to insurance.

PAST MEDICAL HISTORY: Past Medical History

Past Medical History:

Diagnosis

- EGFR-related lung cancer (HCC)
- Headache, chronic migraine without aura
- Long term (current) use of anticoagulants
- Lung disease
- Malignant neoplasm of brain (HCC)

Date

10/2024

03/2025

PAST SURGICAL HISTORY: Past Surgical History

Past Surgical History:

Procedure

Laterality

Date

- HX MYOMECTOMY
- PLEURAL TAP- ORDER TRAY & STERILE GLOVES

2005

10/2024

SOCIAL HISTORY:

Social History

Socioeconomic History

- Marital status: Single
- Spouse name: Not on file
- Number of children: Not on file
- Years of education: Not on file
- Highest education level: Not on file

Occupational History

- Not on file

Tobacco Use

- Smoking status: Never
- Passive exposure: Never
- Smokeless tobacco: Never

Substance and Sexual Activity

- Alcohol use: Never
- Drug use: Never
- Sexual activity: Not on file

Other Topics

Concern

- Not on file

Social History Narrative

- Not on file

Social Drivers of Health

Intimate Partner Violence: Not At Risk (1/24/2026)

Received from University Health

Humiliation, Afraid, Rape, and Kick questionnaire

- Within the last year, have you been afraid of your partner or ex-partner?:

No

- Within the last year, have you been humiliated or emotionally abused in other ways by your partner or ex-partner?: No
- Within the last year, have you been kicked, hit, slapped, or otherwise physically hurt by your partner or ex-partner?: No
- Within the last year, have you been raped or forced to have any kind of sexual activity by your partner or ex-partner?: No

Social Connections: Unknown (1/23/2026)

Received from University Health

Social Connection and Isolation Panel

- Frequency of Communication with Friends and Family: Not on file
- How often do you get together with friends or relatives?: Never
- How often do you attend church or religious services?: Never
- Do you belong to any clubs or organizations such as church groups, unions, fraternal or athletic groups, or school groups?: No
- How often do you attend meetings of the clubs or organizations you belong to?: Never
- Are you married, widowed, divorced, separated, never married, or living with a partner?: Never married

Alcohol Use: Not At Risk (1/24/2026)

Received from University Health

AUDIT-C

- Q1: How often do you have a drink containing alcohol?: Never
- Q2: How many drinks containing alcohol do you have on a typical day when you are drinking?: Patient does not drink
- Q3: How often do you have six or more drinks on one occasion?: Never

Tobacco Use: Low Risk (4/16/2026)

Patient History

- Smoking Tobacco Use: Never
- Smokeless Tobacco Use: Never
- Passive Exposure: Never

Financial Resource Strain: Low Risk (1/23/2026)

Received from University Health

Overall Financial Resource Strain (CARDIA)

- How hard is it for you to pay for the very basics like food, housing, medical care, and heating?: Not hard at all

Depression: Not At Risk (4/14/2026)

PHQ-2

- PHQ-2 Score: 0

Stress: No Stress Concern Present (1/23/2026)

Received from University Health

Finnish Institute of Occupational Health - Occupational Stress

Questionnaire

- Do you feel stress - tense, restless, nervous, or anxious, or unable to sleep at night because your mind is troubled all the time - these days?: Not at all

Food Insecurity: No Food Insecurity (1/24/2026)

Received from University Health

Hunger Vital Sign

- Within the past 12 months, you worried that your food would run out before you got the money to buy more.: Never true
- Within the past 12 months, the food you bought just didn't last and you didn't have money to get more.: Never true

Physical Activity: Inactive (1/23/2026)

Received from University Health

Exercise Vital Sign

- On average, how many days per week do you engage in moderate to strenuous exercise (like a brisk walk)?: 0 days
- Minutes of Exercise per Session: Not on file

Housing Stability: Low Risk (1/24/2026)

Received from University Health

Housing Stability Vital Sign

- In the last 12 months, was there a time when you were not able to pay the mortgage or rent on time?: No
- In the past 12 months, how many times have you moved where you were living?: 0
- At any time in the past 12 months, were you homeless or living in a shelter (including now)?: No

Transportation Needs: Unmet Transportation Needs (1/24/2026)

Received from University Health

PRAPARE - Transportation

- In the past 12 months, has lack of transportation kept you from medical appointments or from getting medications?: Yes
- In the past 12 months, has lack of transportation kept you from meetings, work, or from getting things needed for daily living?: Yes

Health Literacy: Adequate Health Literacy (1/24/2026)

Received from University Health

B1300 Health Literacy

- How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?: Rarely

Utilities: Not At Risk (1/24/2026)

Received from University Health

AHC Utilities

- In the past 12 months has the electric, gas, oil, or water company threatened to shut off services in your home?: No

FAMILY HISTORY: Family History

Family History

Problem	Relation	Name	Age of Onset
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- Esophageal Cancer
- Lung Cancer

Father
Paternal
Grandfather

ALLERGIES: Allergies
No Known Allergies

CURRENT MEDICATIONS: Current Medications

Current Outpatient Medications

Medication	Sig	Dispense	Refill
• amlodipine (NORVASC) 5 MG Oral tablet	Take 1 tablet (5 mg total) by mouth in the morning.		
• carvedilol (COREG) 6.25 MG Oral tablet	Take 1 tablet (6.25 mg total) by mouth in the morning and 1 tablet (6.25 mg total) before bedtime.		
• glipiZIDE (GLUCOTROL) 5 MG Oral tablet	TAKE 1/2 TABLET BY MOUTH TWICE DAILY WITH MEAL		
• NIFEdipine (PROCARDIA XL) 30 MG Oral tablet	Take 1 tablet (30 mg total) by mouth in the morning and 1 tablet (30 mg total) before bedtime.		
• ELIQUIS 2.5 MG Oral tablet	Take 1 tablet (2.5 mg total) by mouth in the morning and 1 tablet (2.5 mg total) before bedtime.		
• TAGRISSO 80 MG Oral TABS	take 1 tablet by mouth daily with or without food		
• amoxicillin-clavulanate (AUGMENTIN) 875-125 MG Oral per tablet	in the morning and in the evening. (Patient		

	not taking: Reported on 4/16/2026)
• losartan (COZAAR) 50 MG Oral tablet	Take 1 tablet (50 mg total) by mouth in the morning. (Patient not taking: Reported on 4/16/2026)
• Lazertinib Mesylate (LAZCLUZE) 240 MG Oral TABS	Oral; Duration: 30 Days
• metformin (GLUCOPHAGE) 1000 MG Oral tablet	Take 1 tablet (1,000 mg total) by mouth in the morning and 1 tablet (1,000 mg total) in the evening. (Patient not taking: Reported on 4/16/2026)
• dexAMETHasone (DECADRON) 1 MG Oral tablet	Take 1 tablet (1 mg total) by mouth in the morning. (Patient not taking: Reported on 4/14/2026)
• dexAMETHasone (DECADRON) 4 MG Oral tablet	Take 1 tablet (4 mg total) by mouth in the morning and 1 tablet (4 mg total) before bedtime. (Patient not taking: Reported on 4/14/2026)

No current facility-administered medications for this visit.

REVIEW OF SYSTEMS:

Constitutional: Positive for fever. Negative for chills.

Endocrine: negative. Negative for cold intolerance and heat intolerance.

Eyes: Negative. Positive for vision changes.

Gastrointestinal: Negative. Positive for diarrhea. Negative for constipation, nausea and vomiting.

Neurological: Positive for headaches, memory changes, numbness and speech change. Negative for seizures and focal weakness.

HENT: negative. Negative for hearing changes.

Respiratory: negative. Positive for shortness of breath. Negative for cough.

Psychiatric: Negative. Positive for depression and anxiety. Negative for insomnia.

Genitourinary: Negative. Negative for burning urination.

Musculoskeletal: Negative. Negative for bone pain, muscle pain and muscle weakness.

Hem/Lymphatic: negative. Negative for bruises/bleeds easily.

Skin: Negative. Negative for rash.

Cardiovascular: negative. Negative for chest pain and palpitations.

PHYSICAL EXAMINATION:

Visit Vitals

BP	(!) 187/101	Comment: RT 192/118
Pulse	77	
Temp	97.6 °F (36.4 °C) (Temporal)	
Wt	219 lb (99.3 kg)	
SpO2	99%	
BMI	38.79 kg/m ²	
Smoking	Never	
Status		
BSA	2.1 m ²	

Physical Exam

Constitutional:

General: She is not in acute distress.

Appearance: Normal appearance. She is not ill-appearing.

Eyes:

Extraocular Movements: Extraocular movements intact.

Pupils: Pupils are equal, round, and reactive to light.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Pulses: Normal pulses.

Heart sounds: Normal heart sounds.

Pulmonary:

Effort: Pulmonary effort is normal.

Breath sounds: Normal breath sounds.

Abdominal:

General: Abdomen is flat.

Palpations: Abdomen is soft.

Musculoskeletal:

General: Normal range of motion.

Cervical back: Normal range of motion and neck supple.

Right lower leg: Edema present.

Left lower leg: Edema present.

Skin:

General: Skin is warm and dry.

Neurological:

General: No focal deficit present.

Mental Status: She is alert. Mental status is at baseline.

Cranial Nerves: No cranial nerve deficit.

Sensory: No sensory deficit.

Motor: No weakness.

Coordination: Coordination abnormal.

Gait: Gait abnormal.

Comments: **Ataxia**

Pressured speech

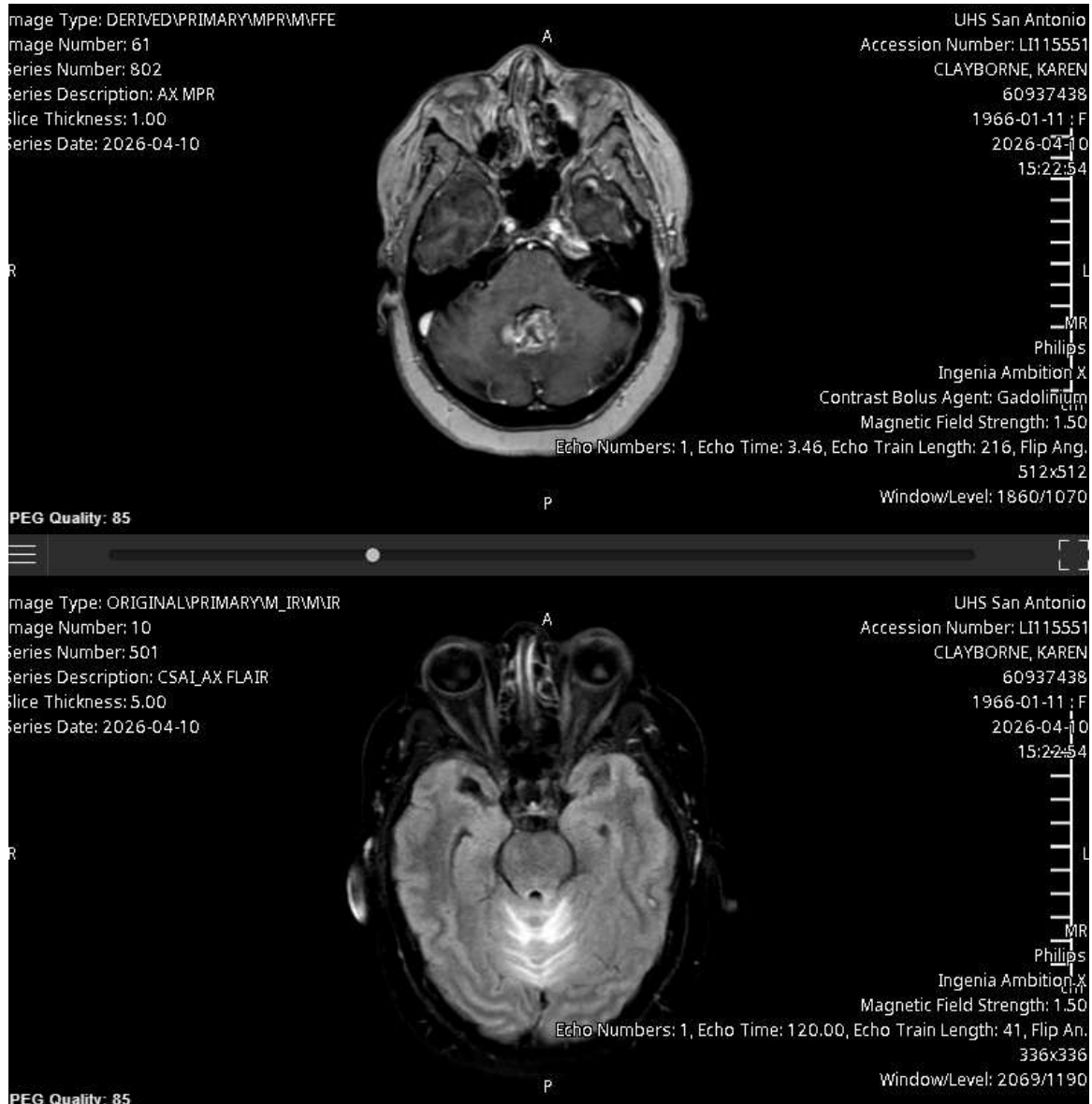
Tangential thought

Psychiatric:

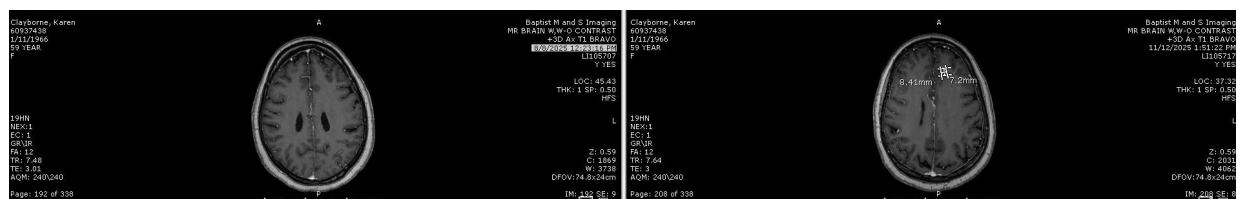
Mood and Affect: Mood normal.

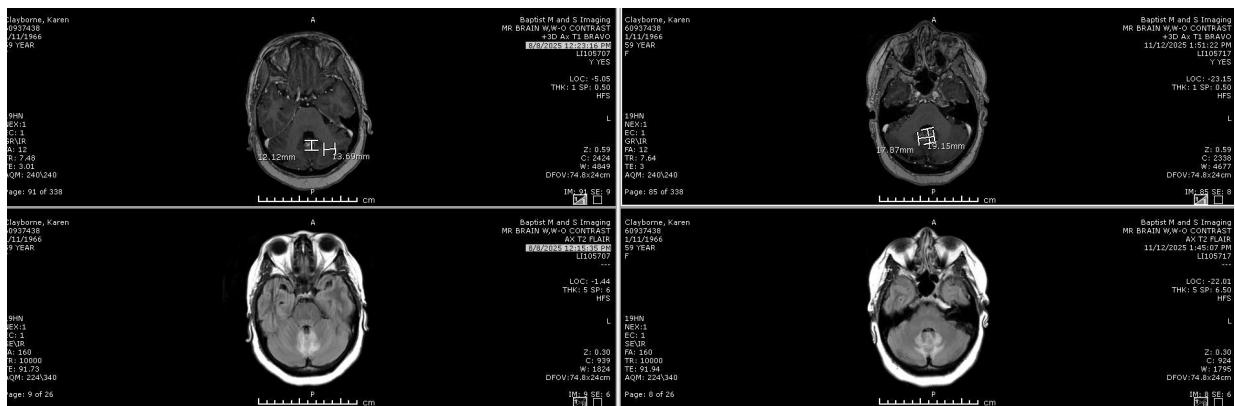
 **No data to display**

DIAGNOSTIC IMAGING: Diagnostic imaging was reviewed and interpreted by myself. MRI Brain 4/10/26 shows interval increase in midline heterogeneous nodular L paramedian vermis/cerebellar lesion, interval more pronounced punctate nodular enhancement in L lateral and lower cerebellum and b/l occipital lobes, decrease in L paramedian frontal lobe enhancing lesion, interval mildly increased size of upper and mid cerebellar vermis.



MRI 11/12/25 shows **increase in cerebellar vermis mass** (1.9x1.7cm from 1.3x1.2cm on 8/8/25) and **new R frontal mass** (0.7x0.8cm). Associated T2/FLAIR hyperintensity.





CT PE 11/7/24

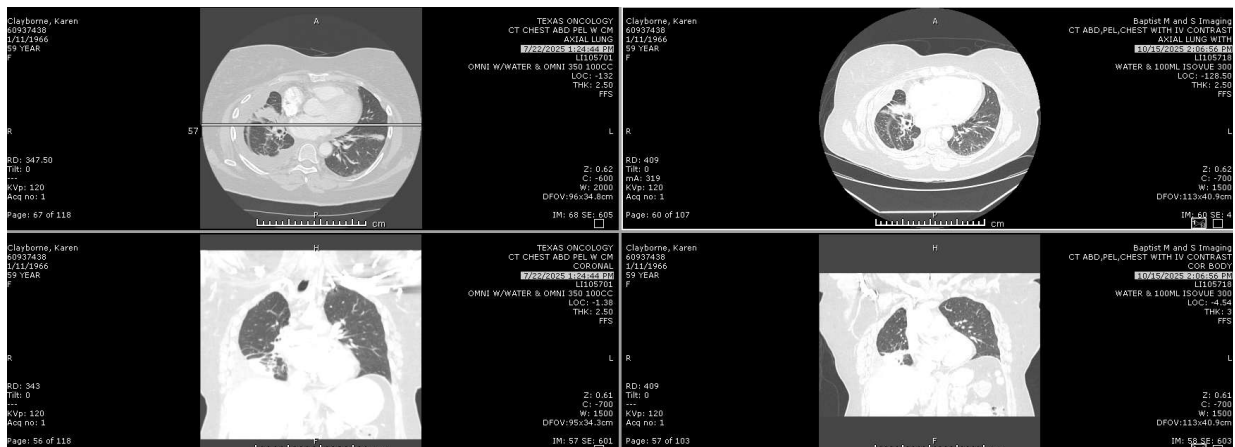
Impression

1. Limited assessment of pulmonary embolism due to suboptimal vascular opacification. No obvious central pulmonary embolus.
2. Large right pleural effusion, resulting in moderate mediastinal shift.
3. Complete opacification of the right lung with obliteration of the right mainstem bronchus, highly suspicious for underlying bronchogenic carcinoma.
4. Mediastinal lymphadenopathy.
5. Osseous metastatic disease at T3 and T9. There is extra vertebral extension of tumor at T9, likely resulting in spinal and neural foraminal stenosis. Correlate with neurological examination. Recommend further evaluation with contrast-enhanced MRI.

CT AP 11/7/24

Impression

1. Osteolytic lesions at T9 and L3 with destruction of the T9 pedicle and suspected spinal canal stenosis at this level.
2. Multiple vague ill-defined hypoenhancing hepatic lesions. Metastatic disease should be considered.
3. Uterine enlargement with suboptimal evaluation for a discrete mass. MRI of the pelvis may be of benefit for more precise characterization.
4. Mild diverticulosis without acute diverticulitis.
5. Please see the accompanying report of the CTA chest performed on today's date, November 7, 2024.



PATHOLOGY:

Collection Date: 09/23/24 Received Date: 09/24/24 Pathology Number: MSO:MC24-1121

Submitting Doctor: Gabino Miranda, Gustavo A MD

CLINICAL HISTORY

Pleural effusion

FINAL DIAGNOSIS

PLEURAL FLUID, RIGHT CHEST THORACENTESIS:

MALIGNANT CELLS ARE PRESENT CONSISTENT WITH ADENOCARCINOMA METASTATIC FROM
A LUNG PRIMARY.

IMPRESSION/RECOMMENDATIONS: Karen Clayborne is a 59 y.o. female presenting with NSCLC

It is unclear from talking with her if she would like to transfer care to UTHSA or get a second opinion. Will reach out and discuss with her primary oncologist. Discussed at his number (2105955300) on 12/10/25. Dr. Alluri is considering SRS and continuing osimertinib. Spoke with Dr. Alluri again 1/14/26. He saw her and is wondering if we can admit her for radiation. The situation is challenging because, while I believe the patient has capacity to make her own medical decisions, she is a poor historian, has extreme difficulty navigating the healthcare system and has paraphasia. Per Dr. Alluri, this predates the worsening brain metastasis. We are both worried that if we continue to try to manage the brain metastasis outpatient that she will be lost to follow up. I had a detailed discussion with her and her family 4/16/26. I explained that both Dr. Alluri and I want what is best for her but it is in her best interest to pick a 'quarterback'. I explained how the lack of continuity is impacting her care. As an example, she was hesitant for me to treat her hypertension 4/16/26, saying she would see another doctor for that.

Headaches:

Brain Metastasis:

- Continue dexamethasone
- Discussing with Dr. Dalwadi

NSCLC:

- Continue osimertinib
- Will likely need social worker as she is switching insurance
- Repeat CT TAP

Skeletal Metastasis:

- Continue Zometa

History of Pulmonary Embolism: She completed 3 months of Eliquis and 1 month of rivaroxaban. Likely needs lifelong AC in setting of malignancy.

- Will likely need referral to hematology
- Continue apixaban

Small bowel perforation: Secondary to sigmoid diverticulitis. Admitted 6/10/25, general surgery consulted determination to conservatively manage.

Germline BRCA2 mutation:

- Will likely need Genetic testing for patient and family

Psychiatric Comorbidity: Multiple providers have noted pressured speech, tangential thought, poor insight which is impairing her ability to navigate the healthcare system. I spoke with the patient and family about this at length 4/16/26. They do not believe she has any problem that warrants psychiatric attention and do not wish to see psychiatry.

- Continue to readdress

HTN:

- Carvediol 6.25 BID
- Losartan 50mg
- Hydralazine as needed for BP >160/110

I spent a total of at least 60 minutes with the patient or documentation, and over half of the time was spent counseling the patient about the status of the disease, response to treatment and prognosis, supportive care measures for pain and symptom management, addressing all concerns, and coordinating follow-up care. The patient may be seen subsequently by Mays advance practice providers (APPs) incident to the diagnoses noted in this encounter. Follow-up diagnostic studies and therapeutic interventions for the expected course of disease and treatment side-effects, based upon commonly accepted standards, guidelines and pathways, will be ordered by the APPs from the treatment plan, available supportive care plans and the approved MCC Preference list.

*William Kelly, MD
Hematology/Oncology
Assistant Professor
UT Health Science Center at San Antonio*

G2211 Attestation: I will continue to be the long-term focal point of care for this patient regarding oncologic care.

